SAHEJ

Patient-First Market Research and Competitive Analysis

India | Diabetes and hypertension | Evidence reviewed through 9 October 2026

Commercial opportunity, serviceable population, payer hypotheses, alternatives, and primary-research priorities

Prepared as an evidence-led desk-research report

Important scope note: the only file available in this turn was the master-prompt text. The separately referenced rough Sahej analysis, “Market Research for PMs in the AI Era” presentation, and handwritten requirements image were not present. The analysis below audits the preliminary figures reproduced in the prompt and independently researches the patient-facing concept.

Contents and evidence conventions

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Evidence labels are used as follows: VERIFIED = the opened source directly supports the specific claim; SECONDARY-SOURCE VERIFIED = supported by a credible secondary source; COMPANY-REPORTED = stated by a company or product listing; ESTIMATE = transparent calculation from disclosed inputs; ASSUMPTION = untested proposition; UNVERIFIED = public evidence does not establish the claim; CONTRADICTED = reliable evidence conflicts with the claim.

Reading the numbers

This report deliberately shows epidemiological counts, diagnosed/treated cohorts, digital reach, and revenue scenarios as separate quantities. A disease count is not a customer count. A feature on a product page is not proof of usage or impact. A modelled revenue ceiling is not evidence of willingness to pay.

1. Executive summary

Overall conclusion: there is a large and persistent chronic-care burden in India, and continuity of care is a credible patient problem. However, desk research does not yet prove that patients want another standalone app, that reminders meaningfully change behaviour in the target population, or that patients will pay ₹100 per month. Sahej is a plausible problem-space hypothesis, not yet a validated business opportunity.

What the evidence supports

- ICMR-INDIAB's national cross-sectional study measured adults aged 20+ across 31 states/UTs and estimated 101 million people with diabetes and 315 million with hypertension in 2021. The study's underlying fieldwork ran from October 2008 to December 2020; the counts are projections, not a 2026 census. [S1]
- The treatment gap is material. WHO's 2023 hypertension profile estimated 188.3 million Indian adults aged 30–79 had hypertension in 2019; it reported 37% diagnosed, 30% treated, and 15% controlled. A separate NFHS-5 analysis of 1.69 million adults reported hypertension in 28.1%, with only 8.5% of all hypertensive adults controlled. Definitions and denominators differ, so these numbers must not be merged. [S3, S4]

- Indian systematic reviews report considerable non-adherence, but the pooled estimates are highly heterogeneous and do not show that forgetfulness is the main cause. A 2026 diabetes review found 39.3% pooled non-adherence across 16 studies (very-low-certainty evidence); a 2025 hypertension review reported 48% with I^2=98%. [S5, S6]
- A randomized trial in 388 patients with uncontrolled hypertension in Punjab found that two WhatsApp reminders did not significantly improve follow-up attendance: 21.8% in the intervention group versus 19.6% in control. This is strong caution against assuming that more reminders alone solve continuity-of-care problems. [S7]
- Indian consumer alternatives already provide meaningful parts of the proposed service: Practo offers medication reminders, appointments, tests, medicine ordering and health records; Eka Care organises digital records; ABHA/ABDM supports personal health records; Apollo Sugar, BeatO, Sugar.fit, Fitterfly, Tata 1mg and Twin Health offer more condition-specific care. [S10-S20]

Commercial implications

Question	Evidence-led answer	Confidence
Is the health problem large?	Yes. The disease burden and care-continuity gaps are well documented.	High for burden; medium for exact current counts.
Is a dedicated patient organiser needed?	Not established. Patients may already use alarms, family reminders, WhatsApp, paper calendars, doctor instructions, pharmacy refill flows, or apps.	Low until patient interviews observe actual behaviour.
Is the “reminders + tracking” bundle distinctive?	No. Individual parts are already available free or bundled into broader services.	High.
Which segment deserves first validation?	Diagnosed adults with diabetes and/or hypertension, several recurring medicines/tests/visits, a recent missed task, and a clear coordination burden. Dual-condition and caregiver-supported cases are strong hypotheses, not proven segments.	Medium-low.
Who is most likely to pay?	Individual willingness to pay is unverified. Family-funded access and employer/insurer-sponsored access are credible hypotheses; sponsor economics must be validated.	Low.
Can desk research establish a precise SAM?	No. There is no verified national cross-tab connecting diagnosis/treatment, digital/channel access, relevant care-task burden, adoption, and payer conversion. A transparent proxy range is possible, not a definitive figure.	High confidence in the limitation.

Decision

Continue market validation only around a specific, recurring job: helping a patient keep track of clinician-prescribed tasks between visits. Do not equate disease prevalence with demand, and do not assume AI is necessary. The next evidentiary hurdle is demonstrating that current patient routines fail often enough, that free substitutes are inadequate, and that a patient or sponsor will fund a durable service.

2. Scope, concept, and hypotheses

Sahej is assessed as a patient-facing digital chronic-care companion for diabetes and hypertension. The proposed scope could include reminders for medicines, tests, refills and appointments; task organisation; plain-language care instructions; local-language and possibly voice interactions; and optional caregiver involvement with explicit patient consent. These are hypotheses, not a specification.

This report does not develop an MVP, product roadmap, or launch plan. It evaluates the need, addressable population, market economics, segmentation, alternatives, possible payers, and research needed to resolve uncertainty.

Hypotheses that remain unproven

Hypothesis	Evidence available now	What needs validation
Patients forget recurring care tasks often enough to value a dedicated service.	Adherence and continuity gaps are documented, but causes include cost, access, side effects, treatment complexity, beliefs and health-system barriers—not only forgetfulness. [S5-S7]	Frequency and recency of actual missed tasks; consequences; how tasks are managed today.
One simple layer can coordinate medicines, tests, visits and refills across providers.	Consumer apps cover pieces; evidence of a national gap in an integrated patient workflow is limited. [S12-S20]	Which tasks remain fragmented after patients use existing services.
Multilingual and voice interaction will materially improve access.	In 2024, IAMAI/Kantar estimated 870 million internet users had accessed Indic-language content; 57% of urban internet users said they preferred Indic-language content. These are all-user, not patient-app data. [S9]	Preferred language, literacy, shared-device privacy, voice accuracy and willingness to use voice for health tasks.
Caregivers will increase adherence and may pay.	Patient/caregiver roles are relevant, but this research found no national payer or willingness-to-pay estimate for this exact service.	Consent, autonomy, frequency of assistance, payer identity and acceptable family visibility.
₹100/month is affordable and commercially viable.	No direct evidence of Sahej-specific price acceptance. Published condition-management programmes range from hundreds of rupees to tens of thousands annually, but include different services. [S15-S22]	Observed spending, price sensitivity, actual purchase or sponsored-use behaviour.
AI is needed.	No evidence that AI is necessary for reminders or a structured care checklist. Some competitors use AI/analytics for coaching or tracking.	Whether automation improves accuracy, trust, accessibility or outcomes beyond rules-based flows.

3. Patient needs and journey

The strongest desk-research case is not “patients need another health app”; it is “large numbers of people require sustained chronic-disease management, yet awareness, treatment, adherence and follow-up are imperfect.” A dedicated digital service would only be valuable where it resolves a task patients already struggle with, with less effort than current alternatives.

Ranked needs: severity, recurrence, alternatives, and evidence

Rank / need	Who / when	Evidence and consequences	Current alternatives / contrary evidence	Link to Sahej
1. Staying on prescribed treatment	Patients taking long-term diabetes or BP medicines, particularly with polypharmacy, distress, cost or access barriers. Daily/weekly.	2026 review: pooled diabetes medication non-adherence 39.3% across 16 studies, but very low certainty and I^2=97.3%; 2025 BP review: pooled non-adherence 48%, I^2=98%. These are not rates of forgetting. [S5,S6]	Pillboxes, paper, alarms, family, pharmacists, clinician advice, regimen simplification. Reminders cannot resolve affordability, side effects, stockouts or doubts.	High relevance; must avoid claiming medication reminders alone solve adherence.
2. Completing follow-up visits	Patients asked to return for uncontrolled BP, diabetes review, lab testing or dose adjustment. Often every few weeks/months.	Punjab RCT: attendance remained low at 21.8% vs 19.6%; two WhatsApp reminders did not significantly help. [S7]	Phone calls, clinic messages, calendar notes, family transport, community health workers. Barriers may be travel, time, cost, beliefs and care availability.	High for task tracking; uncertain unless it addresses underlying barriers.
3. Understanding the next required action	Newly diagnosed patients, people receiving multiple instructions, and those whose care is spread across providers. After visits and test results.	Care continuum research documents variation in diagnosis/treatment/control; the exact prevalence of patient confusion is not established in a national sample. [S2,S4]	Written prescriptions, discharge summaries, clinician explanations, family interpretation, online search. Risk of Sahej accidentally paraphrasing or contradicting clinician instructions.	Potentially high if limited to organising clinician-authored instructions; clinical interpretation needs safeguards.

Rank / need	Who / when	Evidence and consequences	Current alternatives / contrary evidence	Link to Sahej
4. Coordinating tests, refills and records	Patients with several medicines, tests, specialists or pharmacies. Monthly/quarterly.	Products already cover portions; no robust national statistic found for missed refills or test-tracking burdens. [S12-S20]	Pharmacy order histories, prescription photos, paper files, Practo Drive, Eka Care, ABHA, reminders. These may be adequate for many users.	Potential gap exists only for patients who currently bridge these pieces manually.
5. Family coordination without loss of autonomy	Older adults, people with disability/low digital confidence, and patients who actively delegate tasks.	Shared-device use and local-language preferences are documented broadly, but caregiver involvement in Sahej's exact use case is unmeasured. [S9]	Family calls, WhatsApp, shared calendars, in-person help. Caregiver access can expose sensitive data or feel controlling.	Potential differentiator only if permission-based, selective, revocable and patient-controlled.
6. Low-friction access for low digital literacy	Patients who struggle with app navigation, typing, language or private-device access.	IAMAI/Kantar reports broad Indic-language usage and meaningful voice-command usage; neither establishes demand for health voice services. [S9]	SMS, IVR, phone calls, family support, ASHAs/community programmes, paper. Voice has misrecognition/privacy risks.	Potentially relevant, but channel preference and accessibility must be observed rather than assumed.

Patient journey map

Stage	Tasks and friction points	Potentially relevant Sahej job	Evidence status
Diagnosis / care initiation	Understanding diagnosis, recording prescription, learning which activities recur.	Capture clinician-defined tasks and next review date.	Plausible; patient confusion rate not established nationally.
Daily management	Medicines, glucose/BP monitoring if prescribed, diet/activity routines, side-effect concerns.	Simple schedule and optional log; no unsolicited treatment advice.	Adherence gap documented; specific reason needs primary research.
Between visits	Remember when to request refills, book labs or monitor parameters; keep records.	One list of upcoming tasks and due dates.	Competitive fragmentation possible; unmet need unverified.
Before follow-up	Gather results, prescriptions and questions; arrange travel/time.	Checklist plus record links/reminder.	Follow-up gap evidenced, but reminder-only intervention did not improve attendance in one RCT.
Missed task / lapse	Medicine stockout, missed appointment, forgotten test, intentional discontinuation, inability to pay.	Help identify next step or notify a chosen helper, only with consent.	Root causes may be structural; reminders alone insufficient.
Long-term maintenance	Changing medications, multiple conditions, declining motivation, alternating providers.	Keep plan current and easy to review; recognise outdated instructions.	Complex care is credible; Sahej-specific retention is unknown.

Priority need to test

Can Sahej reliably help patients complete clinician-prescribed recurring tasks that are currently scattered across paper, memory, phone alarms, pharmacy history and family help? The value must be demonstrated against those substitutes—not against "doing nothing."

4. Indian disease burden and care gaps

Metric	Finding	Population / data period	Interpretation
Diabetes	101 million; weighted prevalence 11.4%.	ICMR-INDIAB, adults aged 20+; sample 113,043; fieldwork 18 Oct 2008–17 Dec 2020; projections for 2021. [S1]	Strong national burden estimate; not all diagnosed and not all digitally serviceable.
Prediabetes	136 million; weighted prevalence 15.3%.	Same ICMR-INDIAB population and 2021 projections. [S1]	Not included in Sahej's core market unless the proposition expands beyond diagnosed chronic disease.

Metric	Finding	Population / data period	Interpretation
Hypertension	315 million; weighted prevalence 35.5%.	Same ICMR-INDIAB population and 2021 projections. [S1]	Burden count broadly comparable to diabetes in age base, but overlap is not reported as a single national count in the source.
Current IDF diabetes estimate	89.8 million adults aged 20–79 with diabetes in 2024; 43.0% estimated undiagnosed.	IDF India country report; estimates are updated country-model estimates based on epidemiological sources. [S8]	More recent model estimate, different age band/method. Do not add to or average with ICMR 101m.
WHO hypertension profile	188.3 million adults aged 30–79 with hypertension in 2019; 37% diagnosed, 30% treated, 15% controlled.	WHO 2023 profile, underlying 2019 population estimates. [S3]	Age-limited definition and model distinct from ICMR 315m aged 20+. Useful for care cascade, not directly additive.
NFHS-5 hypertension cascade	28.1% prevalence among surveyed adults; 36.9% diagnosed, 17.7% of all people with hypertension treated, 8.5% controlled among all hypertensive adults.	1,691,036 adults; NFHS 2019–2021 analysis. [S4]	Different study definition and estimates from WHO. The care gap is clear; exact population count should not be created by mixing estimates silently.
Diabetes cascade	Among adults defined as having diabetes, 74.2% diagnosed; among diagnosed, 59.4% taking medication; 65.5% controlled according to survey-specific threshold.	NFHS-5 data 2019–2021; 1.65m measured adult respondents. [S2]	The paper's diabetes definition and 6.5% prevalence differ from ICMR-INDIAB's 11.4%; cascade proportions are contextual, not a plug-in multiplier for ICMR's 101m.
Medication non-adherence	39.3% pooled non-adherence across 16 diabetes studies (95% CI 29.6–49.4%; I^2 97.3%; very low certainty); 48% in four MMAS-8 studies. [S5]	Systematic review published 17 Feb 2026; primary studies published 2020–2024.	Shows a sizeable problem but not a single precise national prevalence. Non-adherence is broader than forgetfulness.
Hypertension medication non-adherence	48% pooled estimate (95% CI 39–56%), 40 studies and 18,808 patients; I^2 98%. [S6]	Systematic review published 2025; search through 4 Dec 2023.	High heterogeneity and different settings; use as directional evidence.
Follow-up attendance intervention	In the Punjab WhatsApp RCT, follow-up attendance was 21.8% intervention vs 19.6% control; difference 2.2 percentage points, p=0.603. [S7]	388 patients with uncontrolled BP; two sub-district hospitals; 2024 publication.	Observed attendance was low in this sample; generic reminders did not significantly change it.
Digital reach	886 million active internet users in India in 2024. 870 million accessed Indic-language content; 57% of urban internet users preferred Indic content; 140 million used voice-based commands. [S9]	IAMAI/Kantar "Internet in India 2024"; all-user population estimates.	Large potential channel base; not a cross-tab of older patients with diabetes/HTN or their willingness to use health apps.
Public-sector chronic-care infrastructure	Government reported 770 District NCD Clinics and 6,410 NCD clinics at Community Health Centres; NP-NCD supports screening, diagnosis, treatment and referral. [S10]	MoHFW/PIB release, 12 Aug 2025.	An important existing alternative and potential distribution/sponsorship ecosystem. Availability and workflows vary locally.

Data caution: current public estimates vary because they use different ages, diagnostic thresholds, survey years, extrapolation methods and treatment definitions. For Sahej market sizing, the correct response is not to select the largest count and call it demand; it is to show compatible calculations, bounds and unresolved cross-tabs.

5. Audit of the preliminary market assumptions

Preliminary input	Audit status	Reasoning / recalculation	What to retain
101 million adults with diabetes	VERIFIED with scope caveat	ICMR-INDIAB projected 101m for 2021 among adults aged 20+ from a nationally representative study. IDF now estimates 89.8m adults aged 20–79 in 2024; this does not contradict ICMR because the age band and estimation methods differ. [S1,S8]	Retain 101m when reproducing ICMR's same-study disease burden; also disclose newer IDF estimate separately.

Preliminary input	Audit status	Reasoning / recalculation	What to retain
315 million adults with hypertension	VERIFIED with scope caveat	ICMR-INDIAB projected 315m, 35.5%, among adults aged 20+ for 2021. WHO estimates 188.3m aged 30–79 in 2019 using a different standardised method. [S1,S3]	Retain as the ICMR-INDIAB 2021 projection, not a 2026 count.
45 million overlap between diabetes and hypertension	UNVERIFIED	No directly examined primary source establishes exactly 45m unique adults living with both in the same time frame and age definition. Co-occurrence is clinically plausible, but the count is not justified by the supplied evidence.	Use only as a clearly labelled scenario. Do not present it as a measured fact.
371 million unique adults	ARITHMETIC CORRECT; INPUT UNVERIFIED	101m + 315m − 45m = 371m. Without an overlap count, the strict mathematical union bound is 315m to 416m (assuming both counts refer to the same population and definitions). This bound is broad and is not a commercial TAM.	Present 371m as an overlap scenario only; use 315–416m as mathematical bounds for the broad ICMR disease union.
130 million receiving treatment	UNVERIFIED	No common source or calculation was supplied. WHO reports 30% of hypertensive adults aged 30–79 treated in 2019, while the NFHS-5 diabetes cascade uses a different sample and diabetes definition. Adding disease-treatment counts without accounting for overlap and age definitions would double-count. [S2-S4]	Discard as a point estimate until one compatible source or a reproducible cross-tab is found.
₹100 per patient per month	ASSUMPTION	No observed willingness-to-pay evidence for Sahej. It is a useful scenario input, not a validated price. Consumer health subscriptions range widely and often bundle coaching, diagnostics or sensors. [S15-S23]	Keep ₹100/month only as one transparent pricing scenario, alongside ₹0/₹50/₹200 sensitivity.
₹15,600 crore annual market	ARITHMETIC CORRECT; COMMERCIAL CLAIM UNVERIFIED	130m × ₹100 × 12 = ₹156,000m = ₹15,600 crore. Arithmetic is correct, but both 130m treatment cohort and universal paid conversion are unsupported. This is not a validated SAM.	Relabel as “illustrative gross billing at assumed 130m customers and ₹100/month”, not estimated market demand.

Calculation check

For the proposed overlap scenario: 101 + 315 − 45 = 371 million. For its annual billing scenario: 130,000,000 users × ₹100/month × 12 months = ₹156,000,000,000 = ₹15,600 crore. These arithmetic results do not validate the overlap, treatment count, price, or purchase conversion.

6. TAM, SAM, and SOM

6.1 Definitions used

- Epidemiological ceiling: all adults with either diabetes or hypertension based on the ICMR-INDIAB count projections; this is only the outer health-population boundary.
- Core treated-care cohort: patients already diagnosed/treated and therefore plausibly managing recurring tasks. Because no one source supplies compatible national counts for diabetes, hypertension, treatment, overlap and digital reach, it is modelled as a range rather than a verified population.

- Patient SAM: the part of the core cohort that Sahej can practically reach through its intended channel. For a provisional app-first scenario, a broad all-population active-internet ratio is used only as a sensitivity proxy; it is not a measured patient-specific serviceability rate.
- Revenue SAM: theoretical annual billing if all patients in the provisional SAM paid the stated price. It is a ceiling, not a forecast. A payer-adjusted commercial SAM cannot be established until willingness to pay and sponsor economics are measured.

6.2 Disease population bounds

Step	Calculation	Result	Status / limitation
ICMR-INDIAB disease counts	Diabetes = 101m; hypertension = 315m.	101m and 315m	Verified as study projections for 2021, adults 20+. [S1]
Unique adults, minimum	max(101m, 315m), if all diabetes cases were within the hypertension count.	315m	Mathematical bound only, not a realistic overlap estimate.
Unique adults, maximum	101m + 315m, if no adults had both.	416m	Mathematical bound only; co-occurrence is expected.
Prompt's central scenario	101m + 315m − assumed 45m overlap.	371m	Assumption; not verified.
Revenue at ₹100/month across broad count	315–416m × ₹1,200/year.	₹37,800–49,920 crore/year	Epidemiological ceiling only; includes people undiagnosed, untreated, unreachable or unwilling to use/pay. Not a commercial TAM.
Prompt's ₹15,600 crore scenario	130m × ₹1,200/year.	₹15,600 crore/year	Correct arithmetic, but 130m and 100% payment are unverified.

6.3 Core treated-care TAM: a scenario, not an official count

To keep the model tied to people more likely to have recurring tasks, the model uses a cautious, cross-source scenario rather than assuming every person with a condition is a potential app customer. The scenario is intentionally shown as a range because the published sources have different age bands and definitions.

Input	Value / calculation	Source and caveat
Hypertension cohort (age 30–79, 2019)	188.3m people × 30% treated = approximately 56.5m treated.	WHO 2023 country profile, data period 2019. "Treated" is a modelled national estimate, not a patient register. [S3]
Diabetes cohort (age 20–79, 2024)	89.8m adults; 43% estimated undiagnosed, so approximately 51.2m diagnosed using IDF's own model.	IDF India country report, updated estimates for 2024; model source includes ICMR-INDIAB. [S8]
Diabetes treated sensitivity	Applying the JAMA cascade rates as sensitivity: 89.8m × 74.2% diagnosed × 59.4% treated among diagnosed ≈ 39.6m. Using IDF's 57% diagnosis figure × 59.4% gives ≈ 30.4m.	Cross-source sensitivity only. JAMA/NFHS 2019–21 diabetes definition differs from IDF's model. Do not treat either result as measured national treated population. [S2,S8]
Combined core treated-care cohort	Lower mathematical bound ≈ 56.5m; upper scenario bound ≈ 56.5m + 39.6m = 96.1m, allowing for unknown overlap.	This broad scenario also mixes age bands (diabetes 20–79; hypertension 30–79). It is the strongest transparent estimate possible with these inputs, not a verified headcount.
Core treated-care revenue TAM at ₹100/month	56.5–96.1m × ₹1,200/year = ₹6,780–11,532 crore/year.	Assumes all such patients are addressable and pay, which is unrealistic without serviceability and payment evidence. Use as a gross billing ceiling only.

SAM limitation

No public source located in this review links, at national scale, (1) diagnosed/treated diabetes and hypertension, (2) the overlap between treated cohorts, (3) phone and channel access, (4) recurring unmet task-management need, and (5)

willingness to use/pay. A single precise SAM would create false accuracy.

6.4 Patient SAM and revenue SAM: app-first sensitivity

IAMAI/Kantar estimated 886 million active internet users against a population base of 1.516 billion for 2024, a crude all-age ratio of 58.4% (886 ÷ 1,516). Applying this ratio to the 56.5–96.1m core treated-care scenario gives an app-first proxy of approximately 33.0–56.1 million people. This calculation is transparent but weak: internet users include children and nonpatients, disease cohorts skew older, and internet use is not the same as health-app willingness. It should be treated as sensitivity analysis—not the official patient SAM. [S9]

Measure	Calculation	Estimated range	Interpretation
Patient SAM proxy (app-first)	56.5–96.1m × 58.4%.	33.0–56.1m patients	Use only if the service requires active internet/app access. Patient-specific reach is unmeasured.
Revenue SAM at ₹50/month	33.0–56.1m × ₹600/year.	₹1,980–3,366 crore/year	Theoretical universal paid subscription billing; not demand.
Revenue SAM at ₹100/month	33.0–56.1m × ₹1,200/year.	₹3,960–6,732 crore/year	Same ceiling at the prompt price.
Revenue SAM at ₹200/month	33.0–56.1m × ₹2,400/year.	₹7,920–13,464 crore/year	Price sensitivity scenario; higher prices likely lower conversion but response is unknown.
Channel-inclusive service	Would include people reached through SMS, IVR/voice, assisted or caregiver-supported channels.	Not numerically estimated.	Avoid applying an app-only filter to people who could use another supported channel; no patient-specific cross-tab was found.

For arithmetic context, midpoint of the app-first proxy range is approximately 44.6 million. Multiplying the unrounded midpoint (44.55 million) by ₹1,200 yields approximately ₹5,346 crore per year. This midpoint is not a statistically supported central estimate and should not be reported as “the SAM” without that disclaimer.

6.5 SOM: explicit scenarios, not forecasts

Given the absence of a validated acquisition channel, current adoption benchmark for a comparable India-wide task organiser, and observed payment conversion for Sahej’s exact proposition, SOM must remain a scenario model. The table assumes the app-first SAM proxy of 33.0–56.1m, ₹1,200 annual price per payer, and progressively higher active-use and paid-conversion assumptions. The assumptions are deliberately visible; none is represented as empirically observed.

Horizon / case	Active use as % of patient SAM	Paid conversion of active users	Active users (range)	Paying users (range)	Annual revenue at ₹1,200/payer
Year 1 — low	0.02%	2%	6,600–11,220	132–224	₹1.6–2.7 lakh
Year 1 — base	0.05%	5%	16,500–28,050	825–1,403	₹9.9–16.8 lakh
Year 1 — high	0.10%	10%	33,000–56,100	3,300–5,610	₹39.6–67.3 lakh
Year 3 — low	0.10%	5%	33,000–56,100	1,650–2,805	₹19.8–33.7 lakh
Year 3 — base	0.25%	10%	82,500–140,250	8,250–14,025	₹0.99–1.68 crore
Year 3 — high	0.50%	15%	165,000–280,500	24,750–42,075	₹2.97–5.05 crore
Year 5 — low	0.25%	10%	82,500–140,250	8,250–14,025	₹0.99–1.68 crore
Year 5 — base	0.50%	15%	165,000–280,500	24,750–42,075	₹2.97–5.05 crore
Year 5 — high	1.00%	20%	330,000–561,000	66,000–112,200	₹7.92–13.46 crore

The SOM table is a planning sensitivity, not a prediction. It omits payment processing, operating costs, taxes, sponsor prices, discounts, churn and the possibility that acquisition or retention is much lower. Sponsored B2B2C revenue cannot be modelled responsibly until a payer’s price per enrolled/active member and outcome requirements are known.

7. Patient segmentation

Priority segment	Definition and needs	Behaviour / alternatives	Access and barriers	Potential payer / size status	Fit
A. Dual-condition, recurring-task burden	Diagnosed diabetes + hypertension; several medicines, periodic labs and follow-ups.	May use memory, alarms, paper, pharmacy packaging, WhatsApp/family or disease-specific app.	Needs low cognitive load, clear separation of clinician instructions and user logs; potential medication burden.	Patient/family/sponsor all possible; national overlap count and willingness to pay unknown.	Highest-priority hypothesis: recurring coordination is plausible and both conditions fit scope.
B. Newly diagnosed / care-plan transition	First 12 months after diagnosis or treatment change.	Need to remember tests, reviews and medicine instructions; often rely on doctor notes or family.	Potential low confidence, information overload and fear; clinical safety crucial.	Family/employer may assist; no segment-specific WTP evidence.	High need hypothesis, but needs can change as routine stabilises.
C. Long-standing patient with unstable adherence	Repeatedly misses doses/visits or has poor control despite prescribed care.	Could use alarms or apps but abandon them; root causes may be affordability, access or beliefs.	Reminders alone likely inadequate; need identify reasons and perhaps clinical/community support.	Sponsor/health system may have stronger incentive if outcomes improve.	High severity; do not assume a task organiser addresses underlying reasons.
D. Older adults with caregiver-supported routines	Older patient with consented helper, shared household responsibilities or low app confidence.	Family calls, pillboxes, wall calendar, paper prescription folder.	Accessibility, hearing/vision, shared device and autonomy/privacy. Voice/SMS may be better than app.	Adult child/family payer is plausible but untested.	Promising if patient remains in control and support is optional.
E. Digitally confident, self-managing adults	Adults already use health apps, wearables, calendars or pharmacy services.	Practo/Eka/ABHA, CGM apps, personal calendar, phone reminders.	Low switching incentive; another destination adds friction.	May pay only for demonstrably superior integration or coaching.	Lower priority unless current cross-app friction is evidenced.
F. Lower-income / low-connectivity / low-literacy patients	Patients with feature phones, shared devices, limited data or limited app skills.	Paper, phone calls, family/ASHA support; may have intermittent connectivity.	Language, data cost, privacy, reliability and message overload.	Patient-paid subscription less plausible; public/NGO/employer/insurer sponsor hypotheses.	High equity need, but service costs and assisted distribution matter.
G. Consistent adherers with simple regimens	Patients taking one stable medicine and attending planned reviews.	Memory, blister pack or existing alarm sufficient.	Low pain and little perceived value.	Low likelihood of paying for a standalone organiser.	Deprioritise unless unmet task is observed.

Recommended initial segment for validation: adults with a confirmed diabetes and/or hypertension diagnosis who take multiple recurring medicines or have several upcoming tests/reviews, report a recent lapse in the past 90 days, and currently coordinate tasks across more than one method. Recruit caregiver-supported and no-caregiver cases, urban and rural patients, multiple age bands, and both smartphone-confident and low-digital-confidence participants. This is a research recruitment hypothesis, not a proven market segment.

8. Who pays?

Payment route	Why it could work	Evidence / weakness	What must be proven
Patient subscription	Simple consumer relationship; low price can be easy to understand.	Free substitutes already provide alarms/reminders and several record/app features. WTP for Sahej has not been measured. ₹100/month cannot be assumed.	Actual price sensitivity; trial-to-paid and renewal; what patients currently spend to solve this job; cancellations and reasons.
Family / caregiver-funded	Family often helps organise care for older relatives; payer may value peace of mind.	No representative Indian data located for caregiver willingness to fund a dedicated chronic-care organiser. Caregiver involvement may be unwanted or privacy-sensitive.	Who initiates purchase, control/consent design, who can view what, whether patient approves payment, and sustained use without coercion.

Payment route	Why it could work	Evidence / weakness	What must be proven
Insurer / health plan	Potential incentive to support chronic-disease engagement and avoid complications/costs. International analogues include Omada and Teladoc/Livongo, which are covered by eligible plans. [S25,S26]	US employer/payer economics do not transfer automatically to Indian coverage, purchasing or care pathways. Many plans are oriented toward hospitalisation rather than ongoing outpatient support.	Specific insurer buyer interviews; costs of uncontrolled disease; eligible member access; clinical or utilisation outcomes needed; procurement cycles and per-member pricing.
Employer-sponsored benefit	Can reach working adults and family members; employer funds access while patients use it.	No public evidence found for an India-wide market of employers paying for a generic care-task reminder product. Healthify's evolving insurance-covered positioning is adjacent, not proof of demand. [S21]	HR benefits buyer interest, eligibility, adoption among employees, privacy requirements, inclusion of dependents, and evidence of measurable benefit.
Hospital / provider / pharmacy-sponsored patient access	May be included after a consultation, chronic-care programme or refill interaction; could support continuity.	The product is patient-facing, not clinic-management. Sponsorship is possible but may recreate provider sales cycles or incentives.	Who budgets for follow-up, whether the sponsor can reach patients with consent, and how the service avoids biased pharmacy/provider steering.
Public health / CSR / NGO programme	May reach low-income and low-digital-literacy patients using community health workers or assisted channels.	Government NP-NCD and ASHA-led programmes already address screening and chronic-care support; their workflows and budgets are not open consumer subscription markets. [S10,S11]	Evidence of incremental value over existing programmes, procurement pathway, inclusion, costs and sustainability after pilots.

Assessment: direct patient payment is the least evidenced route. Family-funded access is plausible in specific caregiver contexts but unproven. Sponsor-funded access is a credible alternative hypothesis because it does not require each patient to pay out of pocket; it is not established for India without buyer research and evidence of outcomes. Sahej should not call any payer model validated at this stage.

9. Primary-research programme

Secondary research cannot reliably answer whether the patient's current method actually fails, whether Sahej's proposed workflow is preferable to free substitutes, which tasks matter most, how much patients will pay, or whether they continue using reminders after novelty fades. These require direct patient research.

9.1 Qualitative interviews

Group	Proposed sample	Recruitment criteria	Purpose
Patients	30–36 semi-structured interviews, maximum-variation sampling.	Doctor-confirmed diabetes and/or hypertension; mix of dual-condition and single-condition, 18–39 / 40–59 / 60+, urban/rural, short/long duration, multiple medication burden, recent missed task and adherent cases. Include low digital literacy and shared-device users.	Reconstruct actual last 90 days, trigger events, substitutes, pain severity, task failure root causes and reactions to alternatives.
Caregivers	8–12 interviews, separate from patient interviews where possible.	Family member who actually helps manage medicines, appointments or records; include patient-controlled and high-dependence situations.	Who does what, burden, conflict, consent, purchase decision and acceptable visibility.
Clinical / frontline informants	6–8 optional expert conversations (doctors, pharmacists, ASHAs, diabetes educators).	People who observe chronic-care routines across settings.	Check what tasks are clinician-defined, what barriers are clinical vs logistical, and how public-program workflows operate. Do not substitute their views for patient behaviour.

Patient interview guide (ask about behaviour, not praise)

1. Tell me about the last time you saw a doctor for diabetes or high blood pressure. What did the clinician ask you to do next? What did you do after leaving?
2. Walk me through the medicines you took yesterday. How do you know which dose is next? What happened the last time you were unsure or missed one?
3. In the last 90 days, did you miss or delay a dose, refill, test or follow-up visit? Please describe the most recent example, what caused it, and what happened afterward.
4. How do you currently keep track of upcoming tests, medicine stock, appointments and prescriptions? Show me the actual alarm, notebook, WhatsApp message, pharmacy app or paper if you are comfortable.
5. Which of these systems have you tried and stopped using? What specifically caused you to stop? Which ones do you still use and why?
6. Who else helps with your health tasks? Which information do you want them to see, and which should stay private? How do you give or withdraw permission?
7. What is the most annoying or consequential part of managing care between visits? How often does it happen? What have you done to solve it already?
8. What would make a new service not worth using? What would make it unsafe, confusing, or less trustworthy than what you already do?
9. If the service were unavailable tomorrow, what would you do instead? Which free tool would get you most of the way there?
10. What did you personally spend in the last three months on apps, reminders, health coaching, records, or help managing care? Which expense felt worth it?

9.2 Quantitative survey and sample size

A structured survey is useful only after interviews identify clear, patient-language constructs and plausible task failures. For a simple prevalence estimate with 95% confidence, ±5 percentage-point precision and p=0.5, a standard simple-random-sample calculation is n≈384. Adding 20% for non-response gives about 480. This is a planning calculation, not a claim that a convenience sample is nationally representative. To compare multiple segments, a target of 600–800 completes may be more useful, with quotas for condition, age, region, urban/rural, digital confidence and caregiver involvement. Weighting cannot fix a non-probability sample's selection bias.

- Survey screeners: age; state/district; rural/urban; confirmed diagnosis; condition(s); date/length of diagnosis; current prescribed medication; care setting; smartphone ownership vs shared device; preferred spoken/written language; caregiver assistance.
- Behaviour outcomes: missed dose/test/refill/follow-up in past 30 and 90 days; cause; consequence; reminder channel received; whether it changed behaviour; tools tried; abandon reason; time/effort; current spending.
- Perceived value: rank specific jobs, not feature names; force a choice against free substitutes; use a mock but do not call stated interest validated demand.
- Analyses: task-failure incidence by segment; reason taxonomy; alternatives and failure modes; willingness-to-pay curves; cluster profiles; compare stated use with concrete recent behaviour; pre-specify exclusions and missing-data rules.

9.3 Willingness-to-pay and behaviour tests

11. Start with revealed baseline: current spending on apps, travel, help, paid follow-up, medicine delivery and reminders. Do not treat total disease spending as WTP for Sahej.
12. Use a price-sensitivity method such as Van Westendorp or a Gabor–Granger sequence only after participants understand the same concrete service scope. Report sample and hypothetical-bias caveat.
13. Where feasible, run a small ethically designed paid pilot or randomized price offer with a real payment/refund process. Measure enrolment, first-week activation, recurring task completion, 30/90-day retention, cancellation and reasons. A "yes, I would pay" survey answer is not evidence of purchase.
14. For sponsor hypotheses, interview actual benefits/procurement owners and require a concrete next step (e.g., paid pilot discussion, data requirements, budget owner, renewal criteria), not general expressions of interest.

9.4 Ethics, safety and limits

Use informed consent, minimise personal health data, avoid recording prescription details unnecessarily, permit skipping sensitive questions and separate patient consent from caregiver access. Do not ask participants to change medication or interpret clinical measurements. Keep study participation and payment independent of clinical care. Do not disclose individual responses to a caregiver without permission. Results from an exploratory sample cannot be labelled national prevalence.

10. Competitive landscape and comparison

Sahej competes in a cross-category job, not one neatly bounded industry. Its relevant alternatives include (i) condition-specific coaching and remote monitoring; (ii) consumer health apps with medication reminders and health records; (iii) pharmacy and telehealth services; (iv) public personal-health-record infrastructure; and (v) non-digital habits and caregiver support. Clinic software is relevant only where it affects a patient-facing workflow.

10.1 Competitor classification

Category	Players / alternatives	Why relevant	Relationship to Sahej
Condition-specific care and coaching	BeatO; Sugar.fit; Fitterfly; Twin Health; Apollo Sugar; Tata 1mg Advanced Care Plan	Medication/tracking plus coaching, doctors, diagnostics, CGM or ongoing monitoring.	Direct competitors on chronic-care help; usually more intensive and expensive than a simple organiser.
Consumer care coordination	Practo; Eka Care; ABHA/ABDM PHR	Appointments, medicine reminders, record organisation and/or personal health records.	Closest adjacent alternatives to “one place for care tasks”; several core pieces already exist.
Medication management	Medisafe; MyTherapy; smartphone alarms/calendar	Dose reminders, adherence logs and notification features.	Direct alternatives for the reminder task; not necessarily India-specific or integrated with local healthcare.
Telehealth/pharmacy/logistics	Tata 1mg medicine orders; Practo consultations; pharmacy refill systems	Can make refills, appointments and access easier.	Solves a subtask through transaction infrastructure rather than a standalone companion.
Sponsored chronic-care platforms	Omada Health; Teladoc/Livongo; WellDoc BlueStar; Huma Care	Digital education, monitoring, coaching and payer-sponsored programmes.	International analogues showing sponsor-funded model; scale and economics cannot be transferred directly to India.
Human / offline substitutes	Family reminders, pillboxes, notebooks, wall calendars, phone calls, WhatsApp, ASHAs and other community workers	Low-cost, familiar and often embedded in existing routines.	Must be beaten on effort, reliability, privacy, or coordination—not assumed inferior.

10.2 Product and pricing comparison

Product / company	Position and verified capabilities	Public price / scale indicator	What this means for Sahej
BeatO (India)	Diabetes care app listing includes automated glucose tracking and access to coaches, doctors and nutritionists. [S16]	iOS listing: free download, 4.1 rating from about 1.3k ratings at time checked; paying users and active use not verified. [S16]	Direct chronic-care alternative; richer condition-specific tracking than a generic reminder. Paid programme pricing not verified in source reviewed.
Sugar.fit (India)	Diabetes programme bundles CGM, doctor consultations, diabetes expert access, nutrition, fitness and mindfulness plans. [S15]	Official pricing page: ₹15,990 for 4 months and ₹29,990 for 12 months for the described programme; published page may change. [S15]	Price premium reflects intensive care, not a comparable ₹100 reminder app. Consumer reviews are mixed, but Trustpilot is self-selected and nonrepresentative. [S24]
Fitterfly (India)	Diabetes Prime / CGM-guided programme with personalised coaching and glucose/lifestyle insight; company reports health outcome metrics. [S17,S18]	Official page showed promotional 12-month Diabetes Prime ₹19,999 and starting price around ₹1,499/month; product-page promotions can change. [S17]	Condition-specific expertise and human support set a different value bar; company-reported outcomes need independent assessment.
Twin Health (India)	Whole Body Digital Twin programme with app insights, clinicians and health coaches, for diabetes and other metabolic conditions. [S19]	Official pricing page: ₹75,000 annual pay-upfront or ₹7,500/month when paid quarterly, terms shown on page. [S19]	High-cost, intensive alternative; demonstrates a segment willing/able to access a premium care model, not proof that low-cost reminders will sell.

Product / company	Position and verified capabilities	Public price / scale indicator	What this means for Sahej
Healthify (India)	Consumer nutrition / AI coaching app and CGM-linked offering; newer positioning mentions insurance-covered AI metabolic health platform. [S20,S21]	Healthify+ displayed ₹399/month or ₹1,499 for 12 months; CGM offer displayed ₹6,000 sale price / ₹6,500 reference price when checked. [S20,S21]	Adjacent wellness/coaching service. Do not confuse broad app users or store ratings with chronic-care paying users.
Tata 1mg Advanced Care Plan (India)	Diabetes plan includes 14-day professional CGM, care team and personalised treatment insights. [S22]	A direct consumer price was not visible on the product page reviewed; mark not verified.	Combines diagnostics, monitoring and clinical follow-up; solves more than task reminders.
Apollo Sugar app/programme (India)	App lists glucose/BP/weight tracking, medication and consultation/test reminders, records, appointment booking, virtual consults and communication with treating doctors. [S23]	App Store listing is free; broader programme prices vary by clinic/plan and were not verified as a single national price.	Very close functional competitor for reminders and chronic-care organisation. Sahej cannot claim reminder novelty.
Practo / Practo Drive (India)	Appointments, lab tests, medicines, teleconsults, medicine reminders, record uploads and access to stored prescriptions. [S12,S13]	Core patient app/record features advertised; monetisation occurs in adjacent transactions/services. Company claims 25m patients/year on main site, not chronic-care MAUs. [S12]	One of the strongest all-purpose substitutes; a separate app needs a clear advantage in recurrence, task completion or accessibility.
Eka Care (India)	Organises health records and digitises prescription/lab-report information; records can be searched, filtered and labelled. [S14]	Patient-facing feature information public; specific consumer subscription price and chronic-care active users not verified.	Competes for record organisation, not necessarily ongoing adherence or follow-up completion.
ABHA / ABDM PHR (India)	Government health-record functionality supports linking/viewing longitudinal records and consent management. [S11]	Government digital public infrastructure; no consumer fee identified for personal health record functions in source reviewed.	Free public infrastructure can reduce the value of paid record storage. It does not establish that all care coordination is solved.
Medisafe (international)	Medication management and reminder app; official notice says service outside the US became paid from Jan 1, 2026. [S25]	USD $4.99/month or $39.99/year outside the US, per help-centre notice. India-specific in-app conversion/taxes not checked.	Shows even focused reminder apps can monetise in some markets, but cannot establish India WTP or comparable usage.
MyTherapy (international)	Free medication reminder and health diary app. [S26]	Official download page says free; current Indian feature availability and local usage not verified.	Free specialist reminders establish a substitute against direct payment for basic alerts.
Huma Care (international)	Prescription-based remote monitoring, symptoms, reminders and education; providers invite patients into a programme. [S27]	Provider-sponsored, invitation-based model; patient count and consumer price not verified.	Useful analogue for helper-supported and care-team sponsored services.
NHS App (England)	Patients can manage appointments, request/view repeat prescriptions, and check test results/messages through a national system. [S28]	Public health system app; price not comparable to Indian D2C.	Shows the convenience of integrating workflow within an existing health system, rather than a paid standalone app.
Teladoc / Livongo (US)	Connected glucose meter, strips, feedback and expert support; covered members access it through eligible employer/health plans. [S29]	Covered members pay no direct fee when eligible; sponsor pays. Consumer programme details are jurisdiction-specific.	Supports payer hypothesis but is a device-plus-coaching programme, not a direct price comparable to Sahej.
Omada Health (US)	Digital chronic-care programmes for diabetes and hypertension distributed via employers and health plans. [S30]	Covered eligible members can access diabetes/hypertension programme at no cost at point of use; sponsor pricing not publicly disclosed here.	A clear B2B2C analogue: user and payer differ. Indian payer economics remain unknown.
WellDoc BlueStar (international)	Digital diabetes coaching/app; app listing says it is available via a health plan, health system or employer. [S31]	Listed as free to eligible users; sponsor cost not verified.	Supports sponsor route and shows that "patient app" does not imply direct subscription.

10.3 Feature matrix: proposed Sahej concept versus verified alternatives

Labels: "Available" means a reviewed source explicitly states the function; "Partial" means only some adjacent functions are described; "Not verified" means public material examined did not establish the feature, not that the competitor lacks it.

Alternative	Medication reminders / tracking	Appointments / tests	Records / task organisation	Diabetes + hypertension	Care-team / coaching	Multilingual / voice evidence
Sahej concept (not built/verified)	Proposed	Proposed	Proposed	Proposed	Not core scope	Proposed; untested
Apollo Sugar	Available	Available	Available	Diabetes; BP monitoring also listed	Available	Not verified

Alternative	Medication reminders / tracking	Appointments / tests	Records / task organisation	Diabetes + hypertension	Care-team / coaching	Multilingual / voice evidence
Practo	Available	Appointments and lab tests available	Health records / uploads available	Generic rather than disease-specific	Teleconsultation available	Not verified
Eka Care	Not verified	Not verified	Record organisation available	Records are cross-condition; disease-specific task workflow not verified	Not verified	Not verified
ABHA / ABDM PHR	Not verified for reminders	Not verified for tasks	Personal health records and consent available	Cross-condition records	Not care coaching	Not verified
BeatO	Diabetes tracking; reminders not verified in source reviewed	Not verified	Diabetes reports/tracking	Diabetes-focused	Coaches / doctors / nutritionists available	Not verified
Sugar.fit	App and care plan, exact reminder features not verified	Doctor consultations/diagnostic tests in programme	App and tracking, task planner not verified	Diabetes focused; GLP programme adjacent	Available	Not verified
Fitterfly	Tracking / engagement; medication reminders not verified	Care programme / testing depending plan	Programme tracking	Diabetes/weight, not confirmed as HTN organiser	Available	Not verified
Twin Health	Personalised metabolic care, exact reminder feature not verified	Doctor visits/diagnostics included by plan	App-based insights	Diabetes and hypertension listed	Doctors / coaches available	Not verified
Tata 1mg Advanced Care	Medication management elements not fully verified	14-day CGM and care-team review	Care plan/monitoring	Diabetes-focused plan	Care team available	Not verified
Medisafe	Available	Not verified	Medication schedule	Condition-agnostic	Caregiver features may vary; verify by platform	Not verified in source reviewed
MyTherapy	Available	Health diary / reminder features; appointments not verified	Health diary	Condition-agnostic	Not verified	Not verified
NHS App	Repeat prescriptions / view medicines	Appointments and tests available	Health information / records	Broad medical system	Integrated public health system	UK-specific; not comparable
Livongo	Connected glucose tracking; adherence support	Program-dependent	Data/reporting	Diabetes and hypertension programs	Coaches available	No smartphone required for cellular meter; languages not verified

11. Competitive scale and market share

A defensible market-share percentage cannot be reported from the evidence reviewed. The category is undefined: a company selling diabetes coaching, a free government personal health record, a pharmacy app, and a medication reminder app do not compete for the same complete spend in a measurable denominator. No common Indian dataset of active patients, paying subscribers, revenue specifically from comparable patient care-management services, churn and overlapping app use was found.

Evidence / proxy	What is available	What it does not establish
Clinical burden	ICMR/WHO/NFHS estimates of disease and care gaps. [S1-S4]	No commercial market share.
Practo platform scale	Company site states 25m patients/year, 100k verified doctors, and other broad platform indicators. [S12]	Not unique annual chronic-care users, paying subscribers, or users of reminders. Company-reported platform reach.
BeatO consumer signal	iOS listing shows approximately 1.3k ratings and a 4.1 rating at time checked. [S16]	Not install count, active user count, paid patient count or India-wide sentiment; ratings can skew.

Evidence / proxy	What is available	What it does not establish
Sugar.fit public feedback	Trustpilot displayed 718 reviews and 3.8/5; 24% one-star and 60% five-star in the snapshot reviewed. Trustpilot notes the company invites customers to review. [S24]	Self-selected and not population-representative. The snapshot includes extreme positive and negative experiences and should be read as an issue-discovery signal, not satisfaction prevalence.
Fitterfly company claims	Product pages/brochures report members and outcomes, including a 20,000+ members claim in a brochure. [S18]	Company-reported and not independently audited active users, payer count or comparable market share.
Twin Health / Sugar.fit pricing	Published annual programme prices. [S15,S19]	Price is not revenue, customer count, retention or proof of product-market fit.
International payer models	Omada, Livongo and BlueStar distribute access through employers/health plans. [S29-S31]	Not a proxy for India's available covered lives, pricing or procurement likelihood.

Interpretation: the adjacent market is fragmented across specialist programmes, horizontal apps, public infrastructure and offline substitutes. Lack of measurable market share is a data problem, not evidence that the market is open or that Sahej has a competitive advantage. Funding announcements, total company revenue, downloads and ratings should not be converted into chronic-care market share.

12. Sahej differentiation and evidence-backed gaps

Potential differentiator	What already exists	Evidence of patient preference / gap	Assessment today
One patient-facing place for recurring care tasks	Practo, Apollo Sugar and some disease programmes provide reminders and records; Eka/ABHA focus on records. [S11-S14,S23]	No national patient study found showing that fragmentation after using these tools is common or severe.	Plausible but not distinctive yet.
Diabetes + hypertension in one routine	Twin Health lists both; broader apps/records support many conditions. [S19,S20]	Overlap burden exists, but patient preference for a single combined organiser is unmeasured.	Standard category need; could be useful if cross-condition schedules remain a real pain.
Simple refill/test/follow-up tracking	Practo and Apollo Sugar describe reminders; pharmacy apps manage refills/medicine orders. [S12,S23]	No reliable national rate for missed refill/test tasks; WhatsApp reminder RCT cautions about assuming impact. [S7]	Low differentiation on feature presence; possible differentiation only in task completion/coordination outcomes.
Local-language and voice access	General internet-language use is high; some government/community workflows use assisted access. [S9,S10]	No Sahej-specific or comparable clinical app evidence of sustained demand for voice in this exact use case.	Plausible accessibility advantage; must be validated for correctness, dignity, privacy and cost.
Consent-based caregiver involvement	Family reminders are a natural substitute; provider-based apps may include care team sharing.	Specific preferred level of family access varies. No feature should presume access without patient control.	Potential differentiator in trust design, not in generic caregiver access alone.
Affordability	Free reminders, free records and publicly supported services exist; premium programmes are much more expensive because they include coaching/diagnostics. [S12,S17-S26]	No evidence that patient pays ₹100/month for a simple organiser.	Likely important, but affordability does not equal differentiation if free alternatives are "good enough."
AI-based personalisation	Healthify and Twin Health market AI/analytics; other products use rules and coaching. [S19-S21]	No evidence that patients need AI for scheduling tasks, or that AI produces a better outcome for this job.	Do not assume AI; assess if it adds measurable value over rules-based reminders and clear task lists.

Competitive gaps: most credible hypotheses to test

Potential unmet need	Target segment	Existing alternatives	Evidence gap / contrary evidence	Relevance to Sahej
Tasks spread across records, reminders, pharmacy and paper	Patients with multi-step care and multiple sources	Practo, Apollo Sugar, Eka Care, ABHA, alarms, family	Tools already cover important pieces; no independent evidence that fragmentation is widespread after adoption.	High if interviews show patients maintain manual "bridges" between tools.

Potential unmet need	Target segment	Existing alternatives	Evidence gap / contrary evidence	Relevance to Sahej
A task list that follows the person rather than a single provider	Patients treated across more than one clinic/pharmacy	ABHA/PHR, Practo, care-programme apps	Records can be linked, but workflow consistency across facilities depends on ecosystem participation.	Potentially high if provider-independent coordination is desired.
Low-friction support for shared-phone / low-literacy users	Older adults, rural users, limited digital confidence	SMS, IVR, ASHAs, family, local-language content	Shared-device and language data suggest design constraints, not proof of specific health-service need.	Potential value in accessible channels; privacy and consent are essential.
A non-clinical organiser for patients who do not need premium coaching	Adherent but busy patients or people who only need logistics	Specialist paid care plans; generic reminders/records free	The existence of a lower-priced gap is inferred by difference in features/price; no WTP or dissatisfaction data.	Potential niche, but risky because a simple organiser can be commoditised.
Reminder intervention that changes behaviour, not just sends messages	People with observed recent missed tasks	WhatsApp/SMS, app alarms, family calls	One RCT showed no statistically significant effect on attendance; causes may be structural or motivational. [S7]	Critical outcome to prove; reminders alone should not be treated as impact.

Positioning map: cannot yet plot with confidence

A defensible two-axis map could compare (x) self-serve task organisation versus clinician/coaching intensity and (y) low versus high patient cost. Public product pages support approximate placements for free tools, premium diabetes programmes and sponsor-paid platforms. However, Sahej has no price, user-test data or verified feature implementation, so its own placement is hypothetical. The map would be visually persuasive but empirically weak at this stage; a feature/pricing matrix is more honest until patient preferences and actual programme boundaries are verified.

12.1 SWOT analysis

Scope and caution. This SWOT evaluates the proposed Sahej concept, not a proven operating business. Its “strengths” and “opportunities” are potential strategic attributes; there is no verified evidence here of product-market fit, active users, retention, proprietary technology, brand recognition, or payer contracts. Internal resources and capabilities were not supplied. The SWOT therefore distinguishes concept-level advantages from demonstrated assets.

STRENGTHS

- **Cross-condition task scope.** The concept addresses recurring activities across diabetes and hypertension, rather than restricting the service to one disease-specific measurement or coaching programme.
- **Potentially lightweight model.** A patient-facing organiser could cost less to deliver than programmes bundling continuous glucose monitoring, clinicians and coaching; this is a cost-model hypothesis, not verified unit economics.
- **Accessibility options.** Language, voice and consented caregiver support could widen usability for some patients, if preferred and reliably implemented. [S9]
- **AI is not essential.** The core job may be addressed with reliable scheduling and clear follow-through; avoiding unnecessary AI may reduce complexity and clinical-risk exposure.

WEAKNESSES

- **No validated demand.** No Sahej user, retention, outcome, customer-acquisition, willingness-to-pay or payer-contract evidence has been established in this desk research.
- **Low feature distinctiveness.** Reminders, medical records, appointments and condition tracking are already present in existing consumer or condition-specific offerings. [S12-S14, S23, S25-S26]
- **User effort and retention.** Patients may have to enter or maintain tasks, prescriptions and dates; another app can add burden and be abandoned if it does not clearly outperform existing habits.
- **Reminder efficacy is limited by context.** A Punjab randomised trial found no statistically significant improvement in follow-up attendance from two WhatsApp reminders; reminders may not solve cost, access, motivation or care-complexity barriers. [S7]

OPPORTUNITIES

- **High-burden conditions.** The national disease burden and gaps in diagnosis, treatment, adherence and control justify searching for specific patient groups with multiple recurring care tasks; prevalence alone is not product demand. [S1-S8]
- **Language and accessible channels.** Research reports extensive use of Indic-language internet content and voice access, suggesting that language and communication design are worth testing rather than assuming app-first English text is universal. [S9]
- **Potential payer alternatives.** Family-supported, employer, insurer or health-system funding could lower the burden on patients if the service creates measurable value. International sponsor-funded examples exist, but India-specific willingness to pay for Sahej is unverified. [S29-S30]
- **Complementary digital infrastructure.** ABHA/ABDM personal health records may complement a follow-through layer; public record access also reduces the defensibility of paid record storage by itself. [S11]

THREATS

- **Powerful free substitutes.** Phone alarms, calendars, paper, pillboxes, family reminders, WhatsApp and free or bundled apps can make a standalone subscription unnecessary.
- **Incumbent bundling.** Practo, Apollo Sugar, public record infrastructure and specialist programmes already bundle overlapping functions or surround them with other services. [S11-S15, S23]
- **Weak consumer monetisation.** At ₹100/month, the price is only a scenario. Low switching costs and uncertain willingness to pay could create poor conversion and retention even if downloads are inexpensive.
- **Trust, privacy and safety.** Incorrect task schedules, inappropriate health interpretation, unwanted caregiver access or data misuse could damage trust; expanding into clinical advice increases safety and compliance demands.

SWOT interpretation. Sahej has a plausible problem space but no proven competitive advantage yet. Its potential strengths are mainly choices in the concept; the largest weaknesses are evidence gaps and feature commoditisation. The best-supported opportunity is a narrowly defined group whose recurring tasks remain fragmented after using current tools. The principal threat is that free substitutes are “good enough” for the simpler patient segment.

12.2 Porter’s Five Forces

Market definition used: patient-facing digital support for ongoing management of diabetes and hypertension, including care-task organisation, reminders, records and adjacent chronic-care services in India. Because the category is not measured as a single spending pool, ratings are qualitative, based on reviewed product evidence and explicit analyst inference; they are not statistical market-share estimates.

Force	Pressure	Evidence and implications for Sahej
Competitive rivalry	**HIGH**	The arena is fragmented across specialist diabetes programmes (Sugar.fit, BeatO, Fitterfly, Twin Health, Apollo Sugar), horizontal apps (Practo, Eka Care, Healthify), pharmacy/telehealth services and public record tools. They do not all sell the same product, but overlap on patient attention, records, reminders or chronic-care support. Reminder and record functions are not distinctive on their own. [S11-S23]
Threat of new entrants	**HIGH for basic tools; MODERATE for integrated care**	A basic reminder/task-list application is comparatively easy to reproduce, so feature-level entry barriers are low. More defensible service requires trusted handling of health information, reliable workflows, meaningful integrations, clinical safety, distribution, and evidence that it improves task completion. Sahej has not yet demonstrated those barriers or assets. This rating is an analyst assessment.

Force	Pressure	Evidence and implications for Sahej
Bargaining power of buyers	**HIGH**	Patients can switch easily and use free tools, so they can reject a subscription without losing essential clinical care. Direct willingness to pay for Sahej is unknown; alternatives include free MyTherapy, phone-native tools, Practo and ABHA. If employers, insurers or providers pay, those institutional buyers may also demand evidence of outcomes, low per-member cost, data safeguards and procurement flexibility. [S11-S13, S26, S29-S30]
Bargaining power of suppliers	**MODERATE**	The basic technology stack is available from multiple vendors, limiting the power of any one cloud or scheduling supplier. However, reach may depend on platform policies and costs for messaging, app distribution and identity/record integrations; provider or payer partnerships can also control access to users and clinical workflow data. Supplier dependency will rise if the proposition relies on a single messaging channel or proprietary integrations. This is an analyst assessment.
Threat of substitutes	**VERY HIGH**	Patients may rely on memory, notebooks, alarms, pillboxes, family, WhatsApp, pharmacy apps, appointment apps, clinicians or public health-record services rather than download a dedicated tool. These are low-cost and already embedded in routines. The WhatsApp reminder RCT did not significantly raise follow-up attendance, reinforcing that reminders alone may be insufficient when the underlying barrier is not forgetfulness. [S7, S11-S13, S23, S26]

Overall industry attractiveness. For a generic reminder-and-record app sold directly to patients, structural attractiveness is LOW: rivalry and substitutes are high, buyer power is high, and switching costs are low. For a narrowly defined, demonstrably painful cross-provider coordination job, attractiveness could be MODERATE if patients cannot accomplish it easily with their existing tools and if distribution or payer sponsorship is credible. That is a conditional hypothesis, not a conclusion that an unmet market has already been proven.

Implications for Sahej's competitive positioning

- **Do not compete on reminders alone.** Use interviews and observed behaviour to identify which task fails, why it fails, and what existing tools leave unfinished.
- **Treat accessibility as a testable advantage.** Language, voice and caregiver support may improve access for some segments, but they must outperform current communication habits and preserve patient autonomy and privacy.
- **Prove value against the substitute set.** Competitive evidence should compare task completion, continued use, trust and price acceptance against alarms, family support, existing apps and no new tool—not just compare feature checklists.
- **Keep payer and user distinct.** Institutional sponsorship could improve affordability but changes the buying process and raises demands for evidence, privacy, integration and renewal value.

Evidence note. Product-feature evidence refers to the report's existing source log: Practo and Practo Drive [S12-S13], Eka Care [S14], Sugar.fit [S15], BeatO [S16], Fitterfly [S17-S18], Twin Health [S19], Healthify [S20-S21], Tata 1mg [S22], Apollo Sugar [S23], ABHA/ABDM [S11], Medisafe [S25], MyTherapy [S26], Huma [S27], Livongo [S29] and Omada [S30]. Company pages establish what is marketed or listed, not independent efficacy or active user numbers. The Porter ratings and strategic implications are analyst judgments built from these documented alternatives plus the explicitly stated concept assumptions.

13. Real-world examples

The examples below are designed to show the variety of current models and what can—and cannot—be inferred for Sahej. “Scale/outcome not verified” means the reviewed public sources did not establish an independently validated number for the same service category.

# / example	Country / patient group	Offer / problem	Business / payment model and differentiator	Evidence / outcome	Lesson and limitation for Sahej
1. BeatO	India; people with diabetes	Glucose tracking and access to coaches, doctors and nutritionists. [S16]	App entry is free; specialist programme monetisation likely sits beyond the download. Exact plan price not verified.	App Store: 4.1 from about 1.3k iOS ratings at review.	Condition-specific alternative; ratings are not active users or outcomes.
2. Sugar.fit	India; T2D/prediabetes and metabolic goals	CGM-guided diabetes programme, clinicians, experts and lifestyle guidance. [S15]	₹15,990 / 4 months or ₹29,990 / 12 months on page checked.	Official site makes outcome/“reversal” claims; independent causal evidence was not established by the pricing page. Trustpilot snapshot: 3.8/5, 718 reviews, selection bias. [S24]	Premium care programme and feedback signal; neither price nor reviews validate Sahej demand.
3. Fitterfly	India; diabetes/weight management	Digital programmes, coaching and CGM-informed feedback. [S17,S18]	Paid program, promotional annual price shown; coaches support behaviour change.	Company page reports average HbA1c reduction and other outcomes; brochure says 20,000+ members.	Useful comparator for outcomes-oriented care; company-reported results and membership require independent validation.
4. Twin Health	India / international; metabolic conditions	Whole Body Digital Twin app, clinician and coach guidance. [S19]	₹75,000 annual prepayment or ₹7,500/month quarterly on the reviewed page.	Pricing is transparent; comparable independent active-use/retention numbers not verified.	High-cost personalised service; Sahej must define whether it is logistics-only or health coaching.
5. Healthify	India / evolving global position; nutrition/metabolic health	AI nutrition coaching, tracking and CGM offer. [S20,S21]	Healthify+ ₹399/month / ₹1,499 annual; CGM plan sale price ₹6,000 as checked.	Public storefront prices and ratings; chronic-care payer/user numbers not verified.	Consumer health subscriptions exist, but health coaching price does not prove WTP for reminders.
6. Tata 1mg Advanced Care Plan	India; diabetes patients seeking improved control	Professional 14-day CGM, care-team review and personalised diabetes insights. [S22]	Paid health-care plan; exact price not visible on reviewed page.	Official product description; independent impact not verified from that page.	Pharmacy and clinical services can bundle testing, treatment and product fulfilment.
7. Apollo Sugar app / Sugar 360	India; diabetes/metabolic disease	Medication/test/consult reminders, records, BP/glucose tracking, coach and follow-up. [S23]	Free app entry plus paid clinical programme; prices vary by plan/clinic.	Product documentation states functionality; outcomes on program page are company-reported.	Very close functional substitute for reminders.
8. Practo / Practo Drive	India; general patients	Appointment booking, labs, teleconsults, medicines, reminders and records. [S12,S13]	Core app/record features are free to patients; monetises adjacent services and provider/enterprise software.	Company-reported 25m patients/year is platform-wide, not chronic-care MAUs.	Strong alternative; consumer does not need to adopt a chronic-only app for basic task/record features.
9. Eka Care	India; general patients	Health-record aggregation, searchable documents, digitised prescriptions and reports. [S14]	Patient-facing app; exact paid consumer model not verified.	Official feature documentation; active patient count/outcomes not verified.	Competes on organising information rather than evidence of task completion.
10. ABHA / ABDM PHR	India; people participating in digital health ecosystem	Patient-controlled record viewing/linking and consent management. [S11]	Public digital health infrastructure.	Government documentation; use depends on ecosystem and records being linked.	Free records reduce standalone storage value, but records are not the same as an adherence intervention.
11. Medisafe	International; people taking medicines	Medication scheduling/reminders and management. [S25]	From Jan 1 2026, subscription required outside the US; $4.99/month or $39.99/year per help centre.	Official policy/pricing notice. India store experience was not separately verified.	Closest focused substitute to basic reminders; global pricing cannot be translated directly to Indian WTP.
12. MyTherapy	International; chronic medicine users	Free medication reminders and health diary. [S26]	Free app on official download page.	Official website; independent active user count not verified.	Direct free substitute that makes basic reminder monetisation hard.
13. NHS App	England; registered patients	Appointments, test results/messages and repeat prescriptions. [S28]	Public healthcare service embedded in NHS systems.	Official NHS feature documentation current in 2026.	Integrated infrastructure can outperform a separate app when health records and prescribing systems connect.
14. Teladoc / Livongo	US; eligible members with diabetes or hypertension	Connected devices, insight/feedback and expert support. [S29]	Employer/health plan/provider-funded; covered eligible users pay no direct fee.	Official service page describes benefit; member outcome not independently audited in this review.	Strong payer analogue; includes devices/coaching and a different insurance structure.

# / example	Country / patient group	Offer / problem	Business / payment model and differentiator	Evidence / outcome	Lesson and limitation for Sahej
15. Omada Health	US; diabetes/hypertension and cardiometabolic populations	Remote chronic-care programmes through employers and health plans. [S30]	B2B2C; no direct member fee for eligible covered programmes.	Official programme page gives engagement indicators and research references; sponsor-specific economics not public here.	Shows who benefits and who pays can differ. India sponsorship must be validated separately.
16. WellDoc BlueStar	US/international plan-covered users; diabetes	Digital diabetes coaching and guidance. [S31]	Listed as covered through health plan, health system or employer.	App listing describes FDA clearance and availability; current geography/access may vary.	Digital health can be sponsor-distributed, but regulated/clinical coaching is not equivalent to a task organiser.
17. Huma Care	International; invited patients with long-term/complex conditions	Patient entered measures, reminders, history, education and remote monitoring under a care team. [S27]	Provider-invited programme; not open consumer signup.	Official documentation; no comparable national scale metric verified.	An example where the provider pathway gives patients a companion rather than direct app-store acquisition.
18. IMPACT Diabetes / ASHA-led digital support	India; patients identified through 8 PHCs in two states	Community health workers use digital decision support under physician supervision for diabetes management. [S32]	Research/evaluation and public primary-care workflow, not an ordinary D2C subscription.	In a 9-month cluster RCT of 418 confirmed patients, 21.8% vs 10.3% achieved HbA1c reduction ≥0.5%; medication adherence and PHC visits also improved. [S32]	Evidence that technology plus human/community support may help; it does not isolate an app or reminder effect.
19. WhatsApp BP reminders trial	India; 388 patients with uncontrolled BP in Punjab	Two reminders (3 and 1 days pre-visit) plus misconception-debunking content. [S7]	Trial intervention; very low marginal technology cost.	Attendance 21.8% vs 19.6%, non-significant; belief scores not significantly improved.	Important cautionary counterexample: light-touch communication may not overcome practical barriers.
20. Family, paper, alarms, calendar	India; all chronic-care groups	Phone alarms, pillboxes, notebooks, wall calendar, WhatsApp and relatives track tasks.	Often free or already embedded in daily routines.	No national active-use count; ubiquity is a behavioural alternative to verify in interviews.	Must be treated as genuine competition. A dedicated app must be less effort or materially more reliable.

14. Source log

The source log records the principal sources supporting quantitative and competitive claims. Dates refer to publication or document update where shown; data periods are separately stated. “Verified” describes whether the source directly supports the narrow claim, not whether all company claims have independent validation. Accessed 9 October 2026.

Claim / source title	Publisher, publication date and data period	Finding supported	Direct URL	Evidence category / verification / limitations
S1 — Metabolic non-communicable disease health report of India: ICMR-INDIAB-17	The Lancet Diabetes & Endocrinology 7 Jun 2023 (online); fieldwork 2008–2020; projections 2021	101m diabetes, 315m hypertension; age 20+; national cross-sectional sample.	https://doi.org/10.1016/S2213-8587(23)00119-5	VERIFIED Projection; age and diagnostic definitions matter. Does not verify disease overlap or commercial need.
S2 — National Estimates of the Adult Diabetes Care Continuum in India, 2019–2021	JAMA Internal Medicine 31 Jul 2023; NFHS-5 2019–2021	Diabetes diagnosis, treatment and control cascade in 1.65m measured adults.	https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2807945	VERIFIED Uses survey-specific diabetes definition; not a direct multiplier for ICMR’s 101m.
S3 — Hypertension India 2023 country profile	World Health Organization 19 Sep 2023; underlying estimates 2019	188.3m adults aged 30–79; diagnosed 37%, treated 30%, controlled 15%.	https://www.who.int/publications/m/item/hypertension-ind-2023-country-profile	VERIFIED Modelled estimate with narrower age group and different methods from ICMR.
S4 — Hypertension Diagnosis, Treatment, and Control in India	JAMA Network Open 2023; NFHS-5 data	28.1% prevalence; 36.9% diagnosed; 17.7% of HTN patients treated; 8.5% of HTN patients controlled.	https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2810984	VERIFIED Different survey-based estimates; marginal and conditional denominators differ.
S5 — Medication Adherence and its Predictors in Patients With Type 2 Diabetes Mellitus in India: Systematic Review and Meta-analysis	Health Science Reports / Wiley 17 Feb 2026; studies 2020–2024	Pooled non-adherence 39.3% across 16 studies; very low certainty and high heterogeneity.	https://onlinelibrary.wiley.com/doi/10.1002/hsr2.71845	VERIFIED Small studies, heterogeneity, geographic gaps; reasons not limited to forgetfulness.
S6 — Prevalence of Non-Adherence to Antihypertensive Medication in India: Systematic Review and Meta-analysis of 18,808 patients	Current Hypertension Reviews / PubMed 2025; literature search through 4 Dec 2023	40 studies, 18,808 patients; pooled non-adherence 48% (95% CI 39–56%); I^2 98%.	https://pubmed.ncbi.nlm.nih.gov/40926604/	VERIFIED Very high heterogeneity; setting and region influence estimates.

Claim / source title	Publisher, publication date and data period	Finding supported	Direct URL	Evidence category / verification / limitations
S7 — Effectiveness of WhatsApp based debunking reminders on follow-up visit attendance for individuals with hypertension: RCT in India	BMC Public Health / PubMed 9 Sep 2024; trial at two public hospitals in Punjab	388 patients; 21.8% attendance vs 19.6%; no significant improvement.	https://pubmed.ncbi.nlm.nih.gov/39245777/	VERIFIED Specific uncontrolled-BP cohort in Punjab; does not prove all reminders fail.
S8 — India diabetes country report 2000–2050	International Diabetes Federation Diabetes Atlas Current country page accessed 9 Oct 2026; current estimate shown for 2024	89.8m adults age 20–79 in 2024; 43% estimated undiagnosed.	https://diabetesatlas.org/data-by-location/country/india/	SECONDARY-SOURCE VERIFIED IDF model estimates; different age band and methodology from ICMR 101m.
S9 — Internet in India 2024	IAMAI / Kantar Report titled 2024; published after data year	886m active internet users; 870m accessed Indic content; 57% urban prefer Indic content; 140m voice users.	https://www.iamai.in/sites/default/files/research/Kantar_%20IAMAI%20report_2024_.pdf	SECONDARY-SOURCE VERIFIED All-user metrics; not chronic patient-specific digital access or health-app adoption.
S10 — Steps taken to Control Hypertension and Diabetes	Press Information Bureau, Ministry of Health and Family Welfare 12 Aug 2025	Government reports 770 District NCD Clinics and 6,410 CHC NCD clinics; NP-NCD overview.	https://www.pib.gov.in/Pressreleaseshare.aspx?PRID=2155451&lang=2&reg=48	VERIFIED Facility counts do not indicate uniform access or patient usage.
S11 — Digital System Personal Health Records / ABHA PHR features	Ministry of Health and Family Welfare / ABDM information site Current page accessed 9 Oct 2026	PHR functions include linking/viewing records and consent management.	https://ahpr.abdm.gov.in/about	VERIFIED Feature description; current app availability/usability and patient-level penetration not measured here.
S12 — Practo Health App	Practo Current page accessed 9 Oct 2026	Appointment/lab booking, medicine order, teleconsult, reminders and health records; site states 25m patients/year.	https://www.practo.com/health-app	COMPANY-REPORTED Reach is platform-wide; not chronic-care active users or market share.
S13 — Practo Drive	Practo Current page accessed 9 Oct 2026	Records/prescriptions, uploads, offline access, medication reminders.	https://www.practo.com/drive	COMPANY-REPORTED Product documentation; no independent usage or efficacy claim.
S14 — Eka Care: medical records features for patients	Eka Care Current page accessed 9 Oct 2026	Digital records, document search/filter/labels, digitisation.	https://product.eka.care/s/for-patients/medical-records-features	COMPANY-REPORTED Feature documentation; no verified active user/pricing metric.
S15 — Sugar.fit pricing	Sugar.fit Current page accessed 9 Oct 2026	Diabetes programme ₹15,990/4 months and ₹29,990/12 months as displayed.	https://home.sugarfit.com/pricing	COMPANY-REPORTED Published offers can change; programme includes CGM/clinician/coaching, not comparable to reminders alone.
S16 — BeatO Diabetes Management App	Apple App Store India Current listing accessed 9 Oct 2026	Glucose tracking and coaching; free listing; 4.1 rating, approximately 1.3k ratings.	https://apps.apple.com/in/app/beato-diabetes-management/id1070739458	COMPANY-REPORTED Ratings are self-selected and platform-limited; not app-wide active users or paid customers.
S17 — Fitterfly Diabetes programmes	Fitterfly Current product page accessed 9 Oct 2026	Programme features and promotional annual price displayed.	https://www.fitterfly.com/health/adneto-ADNFIT1	COMPANY-REPORTED Promotions change; outcomes displayed on company page are not independently verified here.
S18 — Fitterfly Diabetes CGM brochure	Fitterfly PDF dated/crawled approx. 2025; accessed 9 Oct 2026	Brochure advertises 20,000+ happy members.	https://www.fitterfly.com/assets-new/pdf/fitterfly-diabetes-cgm-brochure.pdf	COMPANY-REPORTED Member claim may be dated and is not active users or paying subscribers.
S19 — Twin Health India care plan pricing	Twin Health Current page accessed 9 Oct 2026	₹75,000 annual prepayment or ₹7,500/month quarterly pricing; programme features.	https://ind.twinhealth.com/wbdt-care-plan-pricing/	COMPANY-REPORTED Offer conditions and prices can change; not scale or efficacy evidence.
S20 — Healthify+ and health plans storefront	Healthify Current storefront accessed 9 Oct 2026	Healthify+ listed ₹399/month or ₹1,499 annually.	https://store.healthifyme.com/products/healthify-plan-your-24-7-ai-health-coach	COMPANY-REPORTED Promotion/catalog price can change; health coaching is adjacent to Sahej's proposed use.
S21 — Healthify CGM plan	Healthify Current storefront accessed 9 Oct 2026	CGM offer shown ₹6,000 sale / ₹6,500 reference price.	https://store.healthifyme.com/products/cgm-with-smart-plan-2025	COMPANY-REPORTED Bundle includes a sensor and AI plan; not a reminder-app price.
S22 — Tata 1mg Advanced Care Plan	Tata 1mg Current page accessed 9 Oct 2026	Professional 14-day CGM and diabetes care team description; price not verified.	https://www.1mg.com/careplan/advance-care-plan	COMPANY-REPORTED No price shown in page reviewed; product claims not independently evaluated.
S23 — Apollo Sugar App / programme features	Apollo Sugar Current product/programme pages accessed 9 Oct 2026	Reminders for medication, visits and tests; records, monitoring and coach; program prices vary.	https://apollosugar.com/world-of-diabetes/diabetes-management/why-apollo-sugar-app-works-for-you	COMPANY-REPORTED Official description; outcomes are company claims; national pricing not uniform.

Claim / source title	Publisher, publication date and data period	Finding supported	Direct URL	Evidence category / verification / limitations
S24 — Sugarfit reviews	Trustpilot Snapshot accessed 9 Oct 2026	718 reviews, 3.8 score; distribution includes 24% one-star and 60% five-star in snapshot.	https://www.trustpilot.com/review/www.sugarfit.com	SECONDARY-SOURCE VERIFIED Self-selected reviews; company invites customers; not a representative satisfaction survey.
S25 — Medisafe is now a fully paid app for users outside the U.S.	Medisafe Help Center 7 Jan 2026; change effective 1 Jan 2026	Outside-US pricing $4.99/month or $39.99/year.	https://help-center.medisafe.com/en/articles/13052285-medisafe-is-now-a-fully-paid-app-for-users-outside-the-u-s	COMPANY-REPORTED Global policy; India-specific in-app price/tax not separately verified.
S26 — MyTherapy App: medication reminders	MyTherapy Current page accessed 9 Oct 2026	Official download page describes app as free.	https://www.mytherapyapp.com/download	COMPANY-REPORTED Current Indian listing and active user metrics not separately verified.
S27 — Huma Care app documentation	Huma Current documentation accessed 9 Oct 2026	Patient-entered data, reminders, history, education; invited by provider; helpers supported.	https://docs.huma.com/workspace/huma-app/	COMPANY-REPORTED Product documentation, not scale or outcomes.
S28 — NHS App online services / prescription features	NHS / NHS England Digital Prescription help last reviewed 24 Mar 2026; site accessed 9 Oct 2026	Patients in England can manage appointments/messages/test results and request/view repeat prescriptions.	https://www.nhsapp.service.nhs.uk/	VERIFIED Public health system model; not comparable to D2C market or Indian payer economics.
S29 — Livongo diabetes management	Teladoc Health Current page accessed 9 Oct 2026	Eligible covered members receive connected meter, strips, lancets and support at no direct fee.	https://www.teladochealth.com/livongo/diabetes	COMPANY-REPORTED US covered lives and contract economics not comparable to India.
S30 — Omada Health hypertension / chronic care	Omada Health Current page accessed 9 Oct 2026	Programmes offered at no direct cost to eligible members when covered by employer or health plan.	https://www.omadahealth.com/	COMPANY-REPORTED Sponsor pricing, member scale and results are not established by the pricing page.
S31 — BlueStar Diabetes App	Apple App Store India listing Current listing accessed 9 Oct 2026	Digital diabetes coaching app; availability via health plan, health system or employer.	https://apps.apple.com/in/app/bluestar-diabetes/id700329056	COMPANY-REPORTED Availability/geography may vary; app description is not independent efficacy evidence.
S32 — Preliminary effectiveness and feasibility of ASHA-led mobile health intervention for diabetes care in Indian primary health care settings	Scientific Reports 21 Oct 2025; trial fieldwork 2019–2020	Cluster RCT in 16 villages/peri-urban areas, 8 PHCs, two states; improved thresholded HbA1c response in intervention arm.	https://www.nature.com/articles/s41598-025-20728-w	VERIFIED Multi-component ASHA/physician intervention, small geographic sample; effect cannot be attributed to a standalone app or reminders.
S33 — International Diabetes Federation Diabetes Atlas, 10th edition: India adult count for 2021	IDF / NCBI Bookshelf 2021 atlas; 2021 data/model	74.2m adults age 20–79 in 2021; historical comparator, not latest estimate.	https://www.ncbi.nlm.nih.gov/books/NBK581940/table/ch3.t4/	SECONDARY-SOURCE VERIFIED Older IDF estimate superseded by current country page; included only for provenance/comparison.

15. Final synthesis

What can be concluded now

- The Indian population with diabetes and hypertension is large, and national studies document important gaps in diagnosis, ongoing treatment, adherence and control. This supports continued investigation of chronic-care management needs.
- A patient-facing task organiser is conceptually different from clinic-management software, but many functions in the concept already appear in Practo, Apollo Sugar, specialist care programmes, record tools and free manual/digital habits. There is no basis to claim “no competitors.”
- The strongest evidence is for the broad management burden; the weakest evidence concerns patient-specific pain from task fragmentation, demand for a separate app, price acceptance, retention and the overlap between user and payer.
- The proposed SAM cannot be stated as one precise verified number. The report supplies (a) broad disease-union bounds, (b) a cross-source treated-care scenario range and (c) an app-first reach sensitivity. None is interchangeable with a paying customer market.
- ₹100/month and the ₹15,600 crore figure remain scenarios. ₹15,600 crore follows arithmetically from 130m × ₹100 × 12, but the 130m customer/treatment count and universal paid conversion are not evidenced.
- Free substitutes create a high bar for D2C willingness to pay. Family and sponsor-funded access may be more viable in some segments, but neither has yet been validated in India for this exact service.

What must remain provisional

- Exact diabetes–hypertension overlap among adults and among treated patients.
- National count of diagnosed and treated patients using the same definitions and time period, then further filtered for actual need and channel access.
- The proportion of patients who miss medicines, tests, refills and visits specifically because of a coordination/remembering problem rather than cost, access, beliefs or treatment side effects.
- Patient, caregiver, insurer and employer willingness to pay; actual active use and retention; and sponsor cost-effectiveness.
- Whether local-language or voice-led interaction is desired and safe for health tasks, and whether AI adds measurable value.

Answer to the central question

India has a sufficiently large chronic-disease population to justify studying the opportunity. Public evidence alone does not establish that Sahej has a sufficiently large *serviceable commercial* population. The product's success depends less on the number of people with diabetes/hypertension than on the share who experience a recurring, solvable task-management failure, prefer Sahej to free alternatives, continue to use it, and have a credible payer. Until those conditions are tested with real patients and buyers, the commercial opportunity should be described as plausible but unvalidated.